Capecitabine

Supportive-care review sheet · For clinician individualisation; not a prescription.

capecitabine twice daily on Days 1–14, then no capecitabine on Days 15–21. Follow your individual prescription.

Before treatment

Confirm the indication and treatment aim, DPD testing, FBC, renal/liver profile, oral-treatment capacity and current medicine list. Record the actual starting dose and any previous reductions.

Antiemetic support

Capecitabine is oral minimal-to-low emetic risk. Agree a rescue antiemetic plan using NCCP/local guidance; no parenteral premedication schedule is automatically prescribed here.

Diarrhoea and hydration

Give a written interruption and escalation plan. NCCP describes loperamide 4 mg initially, then 2 mg after each loose stool, maximum 16 mg/day, or local policy. This is a source reference, not a patient-specific prescription. Fever, blood, severe pain, dehydration or persistent symptoms require assessment.

Mouth and hand-foot care

Agree local mouth care, moisturising and measures to reduce friction/heat on hands and feet. Grade ≥2 hand-foot symptoms require a capecitabine pause; grade 3 requires an individually agreed later reduction.

Interactions

Review brivudine (avoid; at least four weeks before capecitabine), warfarin/coumarins (INR monitoring), phenytoin/fosphenytoin (level monitoring) and allopurinol. Check the complete current SmPC and pharmacy interaction resource for all medicines and supplements.

Blood counts and G-CSF

Monitor FBC before each cycle. Distinguish baseline eligibility, current counts, nadir and recurrence. No fixed G-CSF prescription is generated: assess febrile-neutropenia risk, prior episodes and local prophylaxis policy.

Delivery and monitoring

Capecitabine Days 1–14 every 21 days. Confirm tablet strengths, number per dose and the seven-day rest. No ambulatory pump is required.

Before discharge

Use teach-back for meals, tablet strengths, missed/vomited doses, stop-and-call symptoms, contact numbers and next review. Provide the regimen guide, passport and treatment card.

> NCCP table 2: CrCl 30–50 gives 75% of original dose; <30 or haemodialysis is not recommended. The table’s 50–51 gap requires review. Recovery and recurrence govern later doses; never infer a first episode or re-escalate a prior toxicity reduction.

Sources: NCCP 00216 v8 · Capecitabine SmPC sections 4.2–4.5 · NCCP supportive antiemetic guidance

SACTCheck v0.83.4 · Developed by Dr Paul O’Brien · Source check 16 October 2026 · Consultant/pharmacy review pending.